Concordia University

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SOEN 357 - Mini Project

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1. Introduction

1.1. Context

This mini project is a UX/UI case study for **DoseBuddy**, a mobile health companion app. DoseBuddy is designed for people living with chronic health conditions who need to stay organized with daily medication and regular doctor appointments. Even when someone is responsible and trying their best, managing health routines can get tiring and confusing, especially when there are multiple medications, different timing rules, refills to remember, and follow-up visits.

The purpose of this project is to go through a full UX design process and show how user research and design decisions can shape a clearer, more supportive experience. DoseBuddy focuses on helping users set reminders, track medication usage, manage appointments, and communicate with healthcare professionals in one place.

1.2. Problem Statement

Medication and appointment management often breaks down for normal, everyday reasons: people forget, routines change, notifications get ignored, or the information is hard to find when it matters. A common situation is someone wondering, “Did I already take this dose?” or realizing too late that an appointment is coming up.

Many existing tools feel either too basic (just alarms with no context) or too complicated (too many steps, confusing wording, too many screens). Sometimes the experience is spread across different apps, one for reminders, one for a calendar, another portal for messaging, so users end up switching between places. That friction makes it harder to stay consistent. The main problem DoseBuddy addresses is: how can a mobile app reduce the mental load of chronic care and make the routine feel clearer, simpler, and more reliable?

1.3. Design Goals

The design goals for DoseBuddy were set to keep the product realistic, calm, and easy to use. The app should feel predictable and fast, not overwhelming. The main goals are:

- Support medication adherence: easy reminder setup, clear notifications, and quick logging (taken / skipped / late).
- Make info easy to check: users should be able to confirm what they took and when without digging.
- Help users stay on top of appointments: store appointment details, send reminders, and reduce last-minute surprises.
- Enable simple communication: allow users to send a question or update to a healthcare professional without friction.
- Keep it accessible: clear language, readable layout, and flows that work for different levels of tech comfort

1.4. Target Users & Scope

DoseBuddy targets people who manage their own health daily (for example, someone balancing medication routines with a busy schedule) and people who need extra clarity and reassurance (for example, users who prefer larger text, simpler steps, or more confirmation). The project also considers that some users may rely on a family member or caregiver to help organize appointments and medication, even if they are not the main person using the app.

In terms of scope, this report covers the core UX work for the DoseBuddy concept: research approach (and participant feedback if surveys/interviews are used), two personas, journey maps for each persona, a storyboard, a user flow chart, initial sketches, wireframes, a clickable prototype, and a usability testing section showing feedback and design iterations. The focus is on the design process and the reasoning behind decisions, not building a fully functional medical system.

2. User Research

2.1. Research Goals

Before any design decisions were made, the priority was to understand what people actually go through when managing medications and appointments in their daily lives. It's easy to assume the problems are obvious, but the real picture only comes through when talking to people directly.

The research aimed to answer the following questions:

- How do people currently remember to take their medications, and when does that routine break down?
- What causes missed doses? Is it forgetfulness? A busy day? confusion about timing, or something else?
- How do people feel about reminders and notifications? Do they help, or do they just get ignored?
- Would people actually log their doses, and what would make that feel worth doing?
- How do people keep track of appointments today, and what tends to slip through the cracks?
- What would "messaging a healthcare professional through an app" mean to them, and would they use it?
- Are there accessibility or simplicity needs that should shape the design from the start?

The goal was not to validate assumptions already in place, it was to let the patterns emerge from real conversations and use them to ground every design decision that followed.

2.2. Research Methods

Semi-structured interviews were chosen as the primary research method. This format allows for consistency across participants while still leaving room to follow up on something interesting when it comes up. Each participant was asked the same set of core questions, with light follow-ups depending on where the conversation went.

Interview format:

- 10 participants (P1–P10)
- Approximately 6 to 10 minutes each
- Conducted over voice calls or in person
- Short notes were taken during each session, focusing on recurring patterns rather than word-for-word transcription

Core questions asked:

1. Do you take any medication regularly, or do you help someone who does?
2. What helps you remember to take it, and what tends to make you forget?
3. How do you feel about phone reminders and notifications in general?
4. Have you ever been unsure whether you already took a dose or not?
5. How do you keep track of doctor appointments right now?
6. If one app could make all of this easier, what would it actually need to do?
7. Is there anything about health apps, or apps in general, that frustrates you?

Once all interviews were completed, the notes were reviewed and grouped into recurring themes. Some patterns came up across almost every conversation, such as the "did I already take it?" moment and the frustration with notification overload. Those themes were then translated into concrete design implications, which are outlined at the end of this section.

2.3. Participant Demographics

ID	Age	Occupation	Health Context	Tech Comfort
P1	35	Software engineer	Type 2 diabetes, takes 2 daily meds	High
P2	67	teacher	Hypertension, takes 3 meds	Low

ID	Age	Occupation	Health Context	Tech Comfort
P3	28	Graduate student	Anxiety disorder, takes 1 daily med	High
P4	52	Administrative assistant (coop)	Thyroid condition, takes 1 med	Medium
P5	71	Retired	Arthritis & cholesterol, takes 4 meds	Low–Medium
P6	23	Undergraduate student	Asthma, uses inhaler + 1 daily pill	High
P7	45	Elementary school teacher	Migraines, takes preventative med daily	Medium
P8	58	Bus driver	Type 1 diabetes, takes insulin + 2 meds	Medium
P9	39	Dental hygienist (caregiver role)	Manages elderly parent's 5 medications	High
P10	62	postal worker	Heart condition post-surgery, takes 3 meds	Low

Participants were mostly friends and family, people the author was already in regular contact with at the time. Conversations happened informally over a couple of weeks, which made it easy to get honest, unfiltered answers.

2.4. Key Findings

Finding 1: Missed doses happen because routines get disrupted, not because people are careless

Almost every participant described the same pattern in different words: they stay consistent on predictable days, and things fall apart when something changes. P1, a software engineer, mentioned that back-to-back meetings and long focus blocks make it easy to lose track of time and skip a dose entirely. P7 mentioned that when the school day runs long and she skips lunch, the medication she normally takes with food gets forgotten. The issue isn't a lack of effort, it's that fixed-time reminders don't hold up when the day doesn't go as planned.

Finding 2: Nearly everyone has experienced the "wait, did I already take that?" moment

Nine out of ten participants brought this up, either in response to a direct question or before even getting there. For most people it's more annoying than serious, they shake the pill bottle, try to piece together their morning, and eventually make a guess. For P8, though, who manages insulin-dependent diabetes while driving a bus, getting it wrong in either direction has real health consequences. He described it as one of the more stressful parts of his day, and said a simple log he could check in seconds would make a genuine difference.

Finding 3: Notifications are largely being ignored

Seven out of ten participants said they regularly swipe away phone notifications without reading them, including ones they set themselves. P3 put it simply: *"My phone buzzes constantly, I don't even look anymore."* What participants described as actually effective was a notification specific enough to act on immediately, without needing to unlock the phone or open an app. Four participants mentioned this without being prompted, the ideal being something like "Metformin, tap to log" directly on the lock screen.

Finding 4: Appointments don't get forgotten entirely, but the details around them do

None of the ten participants had one reliable system for tracking appointments. P2 uses a paper calendar but sometimes doesn't check it when the week gets busy. P6 adds appointments to her phone's calendar but almost never includes the supporting details, the clinic address, what to bring, or any prep instructions. P9, who manages her elderly father's appointments alongside her own full-time job, described the experience as exhausting: *"I keep it all in my head because there's nowhere to put it all."* The biggest gap wasn't missing the appointment itself, it was everything surrounding it, like preparation and post-visit follow-ups.

Finding 5: Logging only works if it's genuinely fast

This came up consistently throughout the interviews. Participants said they'd be willing to log their doses, but only if it required almost no effort. P4 said she'd do it as long as she doesn't have to think about it. P10 was more direct, he wanted one button and to be done with it. Any

design involving typing, multi-step flows, or navigating away from the home screen to confirm a dose is a design that gets abandoned quickly.

Finding 6: People want a low-pressure way to reach their healthcare provider

Most participants said they'd use some form of in-app messaging, but with clear expectations about what it's for. The appeal was being able to send a short, non-urgent note, asking about a refill, flagging a side effect, confirming something before a visit, without having to call the clinic or wait on hold. P5 summed it up well: *"I just want to leave a message without it being a whole thing."* Nobody expected a fast reply, and nobody wanted it to feel like an emergency channel. Just a simple, calm way to stay in contact between visits.

Finding 7: Accessibility issues with older users are a design problem, not a user problem

P2, P5, and P10 all mentioned struggling with small text, icons without labels, and layouts that assume prior familiarity with how apps work. P2 has been teaching for decades and is perfectly organized in her professional life, her difficulty wasn't about capability, the apps just weren't designed with her in mind. That's a design failure, and it pushed accessibility to the top of the priority list early in the process.

2.5. Design Decisions & Implications

Each finding above pointed toward something specific in the design of DoseBuddy. The table below summarizes how the research translated into concrete decisions:

Finding	Design Decision
Missed doses happen when routines shift	Allow flexible reminder windows (e.g. "remind me between 8–10 AM") rather than fixed-time alarms only
"Did I take it?" causes daily uncertainty	Show a timestamped dose log on the home screen, one glance should be enough to confirm
Notifications get ignored unless actionable	Support lock-screen logging so users can mark a dose as taken without opening the app

Finding	Design Decision
Appointment details slip through the cracks	Add structured appointment cards with fields for location, prep notes, and post-visit reminders
Logging only happens if it's instant	Make "Mark as Taken" the most prominent action on the home screen, one tap, done
Messaging should feel low-stakes	Keep communication simple: a short thread tied to a medication or appointment, no chat-style pressure
Accessibility is a core requirement	Minimum 16px font size, high contrast, labeled icons throughout, no gesture-only navigation

These findings also directly shaped the two personas developed in the next section. One represents a working-age user with an irregular schedule who needs speed and flexibility above all. The other represents an older user who needs clarity, reassurance, and a layout that never makes them feel like they're doing something wrong.

3. Personas

3.1. Context

The two personas below were created directly from the interview themes and participant profiles. Instead of trying to represent “every” user, they capture the two strongest patterns that kept repeating:

1. a user who is **busy and schedule-driven**, where the main risk is missed doses when the day gets chaotic, and
2. a user who needs **clarity and reassurance**, where the main risk is confusion, loss of confidence, and second-guessing.

These personas were used as decision anchors throughout the design. Whenever a feature or screen was added, it had to answer a real need from at least one persona (for example: one-tap logging for Marcus, or clearly labeled buttons and readable text for Noha). This also kept the product focused and prevented the design from turning into a “feature dump.”

3.2. Persona 1 - Marcus

Marcus is a 58-year-old city bus driver from Montreal managing Type 1 diabetes and two other daily medications. His rotating shifts make consistency hard, fixed alarms get dismissed, and hectic mornings mean he sometimes can't remember if he took a dose. He needs everything fast: one tap to log, one glance to confirm, and reminders flexible enough to follow his schedule rather than fight it.

MARCUS FOURNIER

PROFILE

Gender	:	Male
Age	:	58
Education	:	N/A
Occupation	:	Bus Driver
Address	:	Montreal, Qc

> I don't have time to figure out an app on the road. I just need to know what to take and confirm it fast.

BIOGRAPHY

Marcus has been driving city buses for over 20 years. His days start early, run on a fixed schedule, and leave little room for the unexpected. Diagnosed with Type 1 diabetes in his late 40s, he manages insulin timing alongside two other daily medications. He's committed to his health but his rotating shifts make consistency genuinely difficult.

MOTIVATIONS

Marcus wants to stay on top of his diabetes without it taking over his mental energy at work. He knows that getting his insulin timing wrong has real consequences, so his main motivation is having a system that's reliable enough that he doesn't have to think too hard about it, especially on difficult days.

GOALS

- See what's due and what's already taken at a single glance
- Log a dose in one tap, without navigating through screens
- Get reminders flexible enough to adapt to shifting work hours
- Keep appointment details in one place without relying on memory

FRUSTRATIONS

- Fixed alarms that don't account for rotating shift schedules
- Not being able to remember if he took a dose during a hectic morning
- Health apps that feel built for people sitting at a desk, not working on the move
- The anxiety of getting insulin timing wrong having real health stakes

BEHAVIOURS

Marcus checks his phone briefly at the start of each shift. He responds well to short, specific notifications and acts on them immediately if they require only one tap. He won't engage with anything that slows him down or requires reading through instructions. He builds trust with an app by using it for a week and seeing it work consistently.

TECHNOLOGY

Mobile App

NEEDS

- One-tap dose logging from the lock screen
- Flexible time-range reminders
- Quick home screen log access
- Simple appointment tracking with essential details

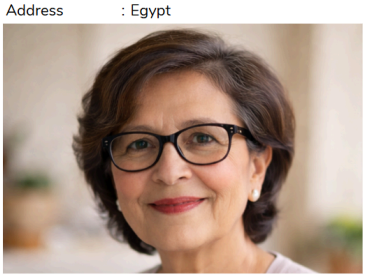
3.3. Persona 2 - Noha

Noha is a 67-year-old recently retired teacher managing hypertension with three daily medications. She's organized and takes her health seriously, but loses confidence in an app the moment something is unclear or unlabeled. Her frustration isn't a lack of ability, most apps just aren't designed with her in mind. She needs large readable text, labeled buttons, and the quiet reassurance that she hasn't missed anything.

NOHA FARAH

PROFILE

Gender : Female
Age : 67
Education : degree in Education
Occupation : part-time substitute, recently retired
Address : Egypt



"I'm not bad with technology, I just need things to be clear. If I have to guess what a button does, I've already lost confidence.

BIOGRAPHY

Noha spent 35 years as a school teacher and brings the same organizational mindset to her personal life. After a hypertension diagnosis a few years ago, she was put on three medications with slightly different timing rules. She takes her health seriously and rarely misses a dose intentionally, but the details sometimes slip, especially in the evening. She uses her smartphone for calls, photos, and WhatsApp, but gets frustrated quickly when an app doesn't explain itself.

MOTIVATIONS

Noha wants to feel in control of her health routine without having to work hard to maintain that feeling. She's not looking for a complicated system, she just wants something that confirms she's on track and makes it easy to catch a mistake before it becomes a problem. Confidence and clarity matter more to her than any advanced feature.

GOALS

Have a calm, readable overview of her daily medication schedule

Feel reassured that she's staying on track without second-guessing

Review her medication history without digging through multiple screens

FRUSTRATIONS

Small text and unlabeled icons that make her distrust an app immediately

Occasionally taking an evening dose twice due to uncertainty about the initial dose.

Apps that are overly cluttered or overly simplified in a condescending manner.

BEHAVIOURS

Noha opens the app in the morning with her coffee to check the day ahead. She reads everything before acting and won't tap something unless she understands what it does. She's likely to abandon an app after one confusing moment rather than troubleshoot it. A clear first-time walkthrough would go a long way in building her trust early.

TECHNOLOGY

Mobile App

NEEDS

- Large, readable text with clearly labeled buttons throughout
- A home screen log she can check instantly to confirm a dose
- Gentle, easy-to-read reminders that are hard to dismiss by accident

4. User Journey

4.1. Context

The user journey maps trace each persona's experience from first discovering DoseBuddy through their ongoing daily use. Each map covers five stages: **Discovery, Onboarding, Daily Use, Managing Appointments,** and **Following Up**. For each stage, the map captures what the user is doing, what they're thinking, how they're feeling, and where the friction is. These maps were used to identify where the design needs to work hardest and where small changes would have the biggest impact on the overall experience.

4.2. Persona 1

Stage	Action	Thoughts	Emotion	Pain Points	Opportunities
Discovery	Hears about DoseBuddy from his doctor, who mentions it during a	"If my doctor recommends it, it's probably worth trying"	Cautiously open	Has tried apps before and dropped them quickly	A doctor referral builds initial credibility, lean into that trust

Stage	Action	Thoughts	Emotion	Pain Points	Opportunities
	routine checkup				
Onboarding	Downloads the app, sets up his medications and shift-based reminders	"This needs to be quick, I don't have time to read through setup screens"	Impatient but willing	Too many steps or unclear options would cause him to abandon setup halfway	Keep onboarding to 3–4 screens max, with smart defaults pre-filled
Daily Use	Gets a lock-screen reminder at the start of his shift, logs his dose with one tap	"Good, that took two seconds"	Relieved, confident	If the notification requires him to open the app, he'll dismiss it and forget	Lock-screen logging is non-negotiable for this persona
Managing Appointments	Checks his upcoming endocrinologist appointment from the home screen	"I just need the time, place, and what to bring"	Neutral	Appointment details scattered across texts and memory cause last-minute stress	A simple appointment card with all key details stored in one place

Stage	Action	Thoughts	Emotion	Pain Points	Opportunities
Following Up	Sends a quick message to his doctor about a side effect he noticed	"I just want to send a note without calling"	Frustrated by the usual process, relieved this is easier	Calling the clinic means hold time he doesn't have on a shift	A low-pressure message thread that doesn't require a phone call

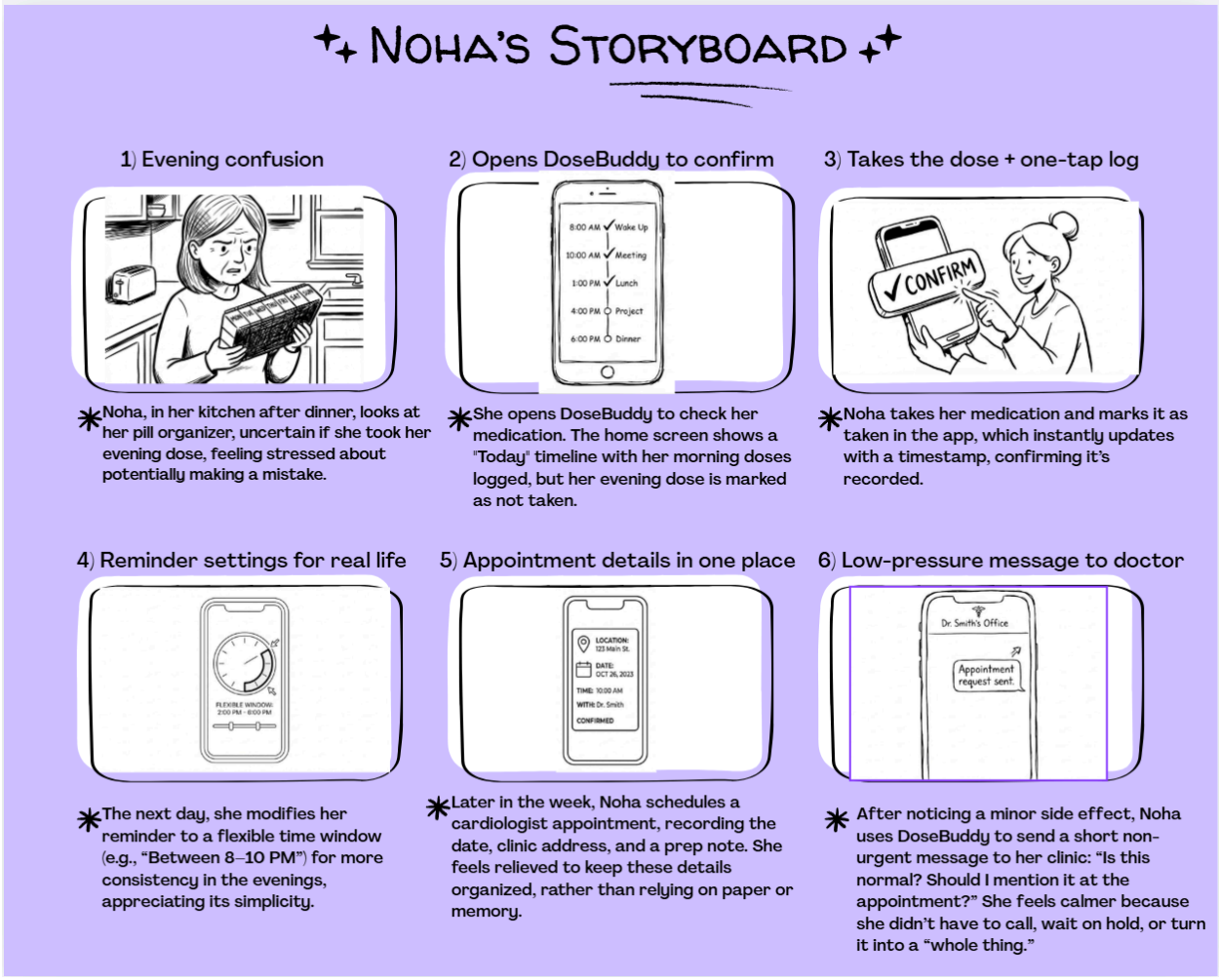
4.3. Persona 2

Stage	Action	Thoughts	Emotion	Pain Points	Opportunities
Discovery	Her daughter shows her the app after noticing she occasionally double-doses in the evening	"I'll try it, but if it's confusing I'm not keeping it"	Skeptical but open	Previous apps felt cluttered or assumed too much prior knowledge	A calm, uncluttered first impression matters more here than anywhere else
Onboarding	Goes through a short walkthrough to set up her three medications and morning reminder	"I need to understand what each step is doing before I tap it"	Anxious, careful	Unlabeled buttons or skippable instructions would cause her to lose trust immediately	A guided walkthrough with clear explanations at each step, no assumptions

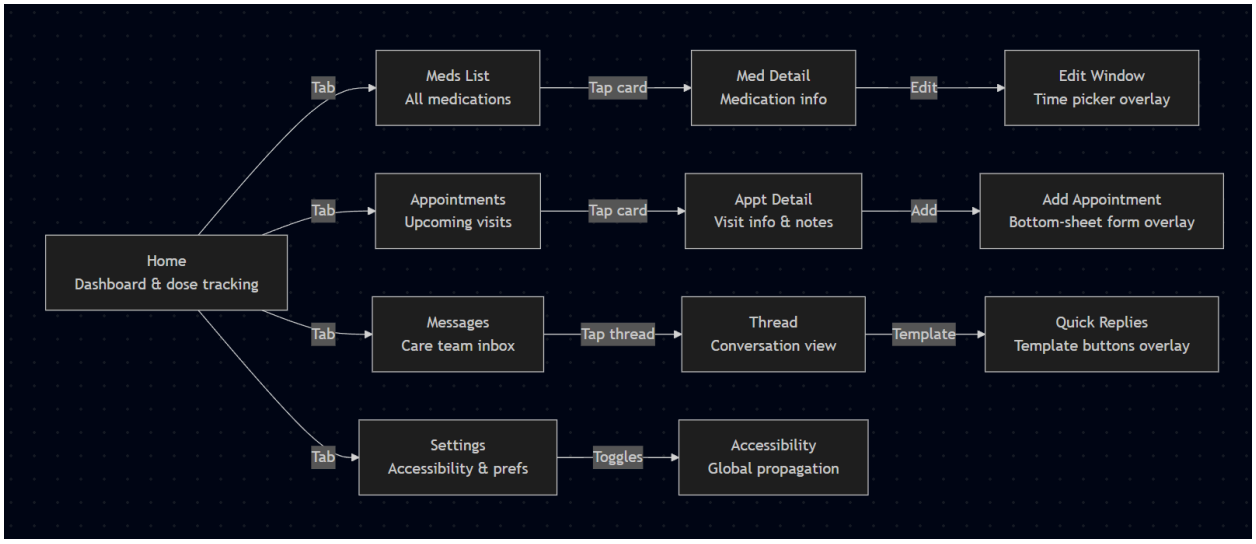
Stage	Action	Thoughts	Emotion	Pain Points	Opportunities
Daily Use	Opens the app with her morning coffee to check her schedule for the day	"I just want to confirm I'm on track"	Calm, reassure d	In the evening, uncertainty about her second dose causes stress	A timestamped log on the home screen that answers the question before she even asks it
Managing Appointments	Stores her cardiologist visit details, date, address, prep instructions, in an appointment card	"This is where I used to lose track of everything"	Relieved	Post-visit notes and follow-up instructions used to end up on loose paper or forgotten entirely	Structured appointment cards with a notes field she can fill in right after a visit
Following Up	Sends her doctor a message asking whether a new symptom is worth mentioning at the next visit	"I don't want to call just for something small, but I do want to say something"	Hesitant, then relieved	The barrier of calling the clinic stops her from raising minor concerns at all	A simple, clearly labeled message thread that feels like leaving a note rather than a formal contact

5. StoryBoard

This storyboard follows Noha through a common situation: she's unsure if she took her evening dose and doesn't want to guess. DoseBuddy helps her confirm it instantly with a home-screen timeline, log it in one tap, and set flexible reminders that fit real life. It also shows how the app keeps appointment details organized and allows a simple, non-urgent message to her clinic when something feels off.

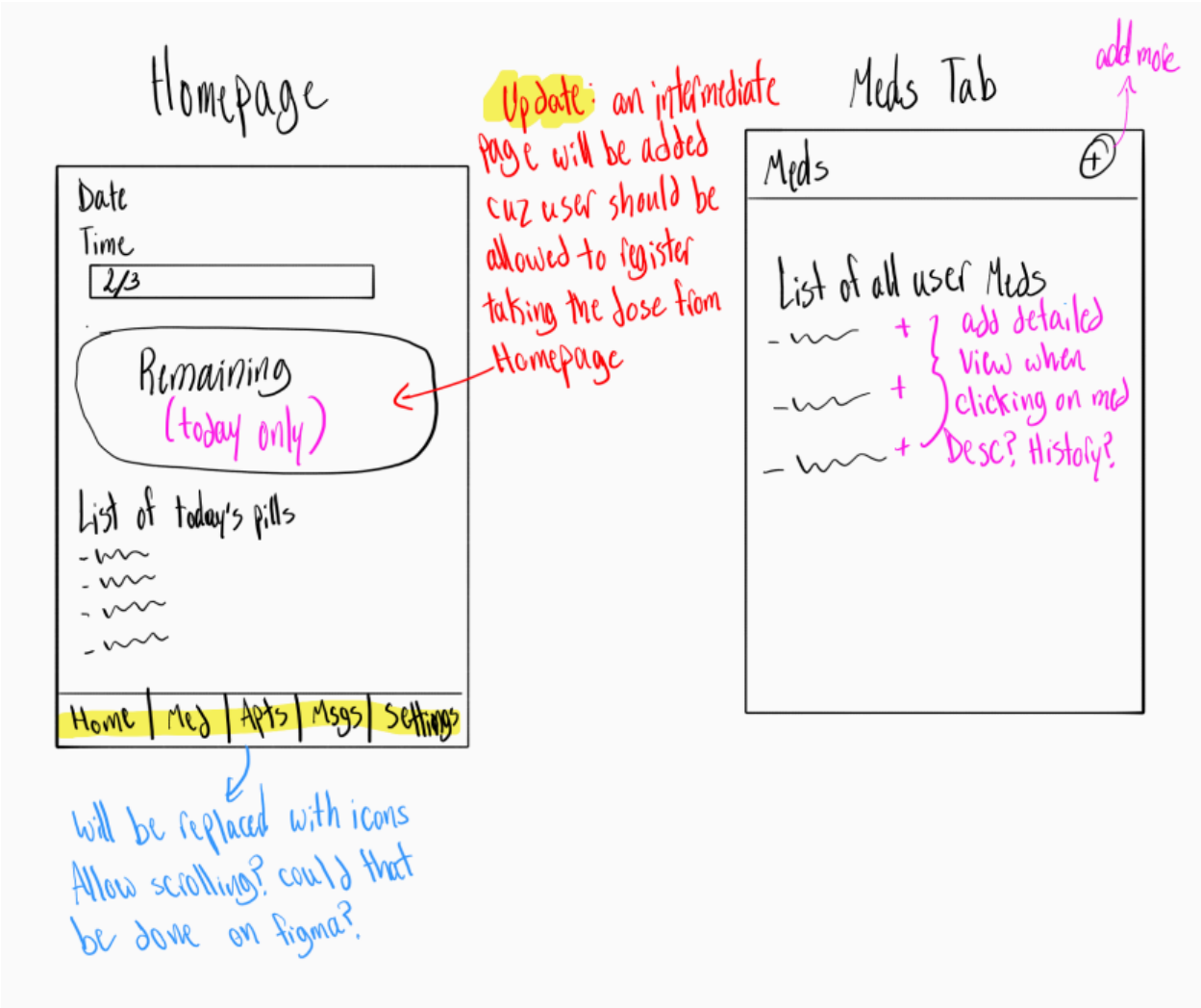


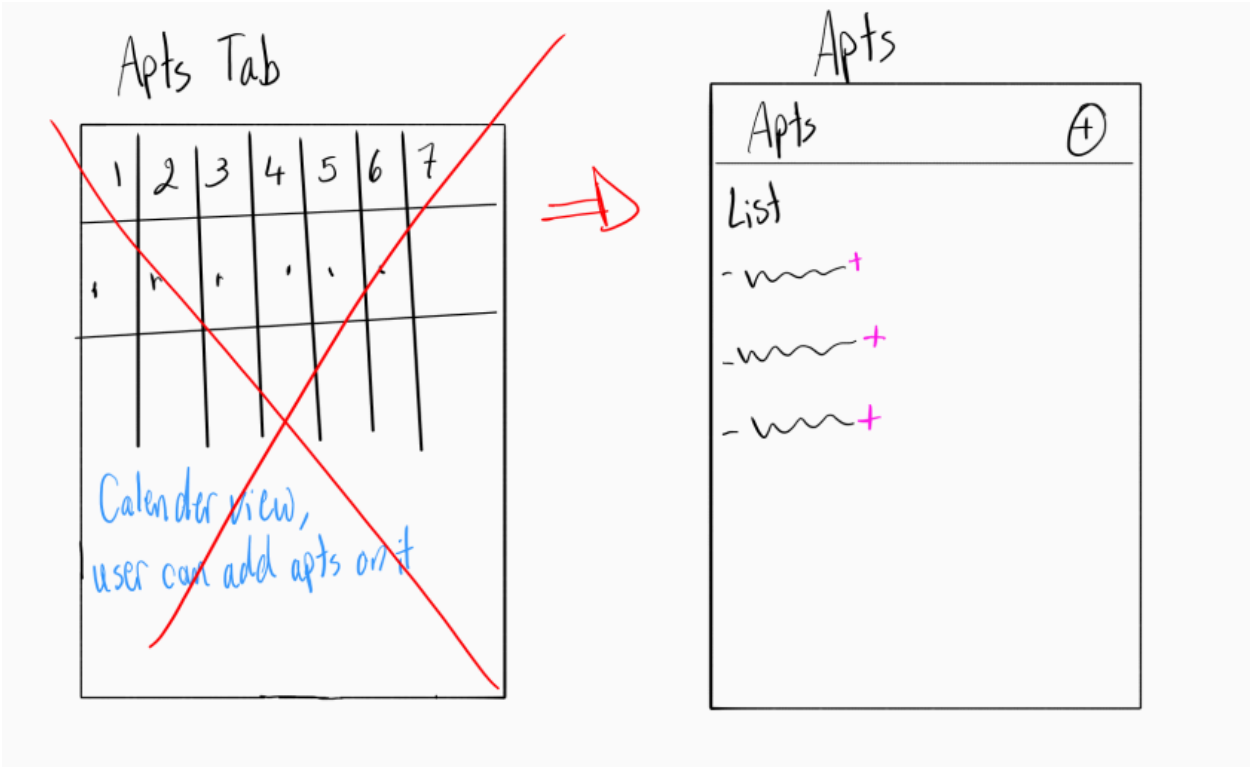
6. User Flow Chart



7. Initial Sketches, Wireframes & Prototypes

7.1. Initial

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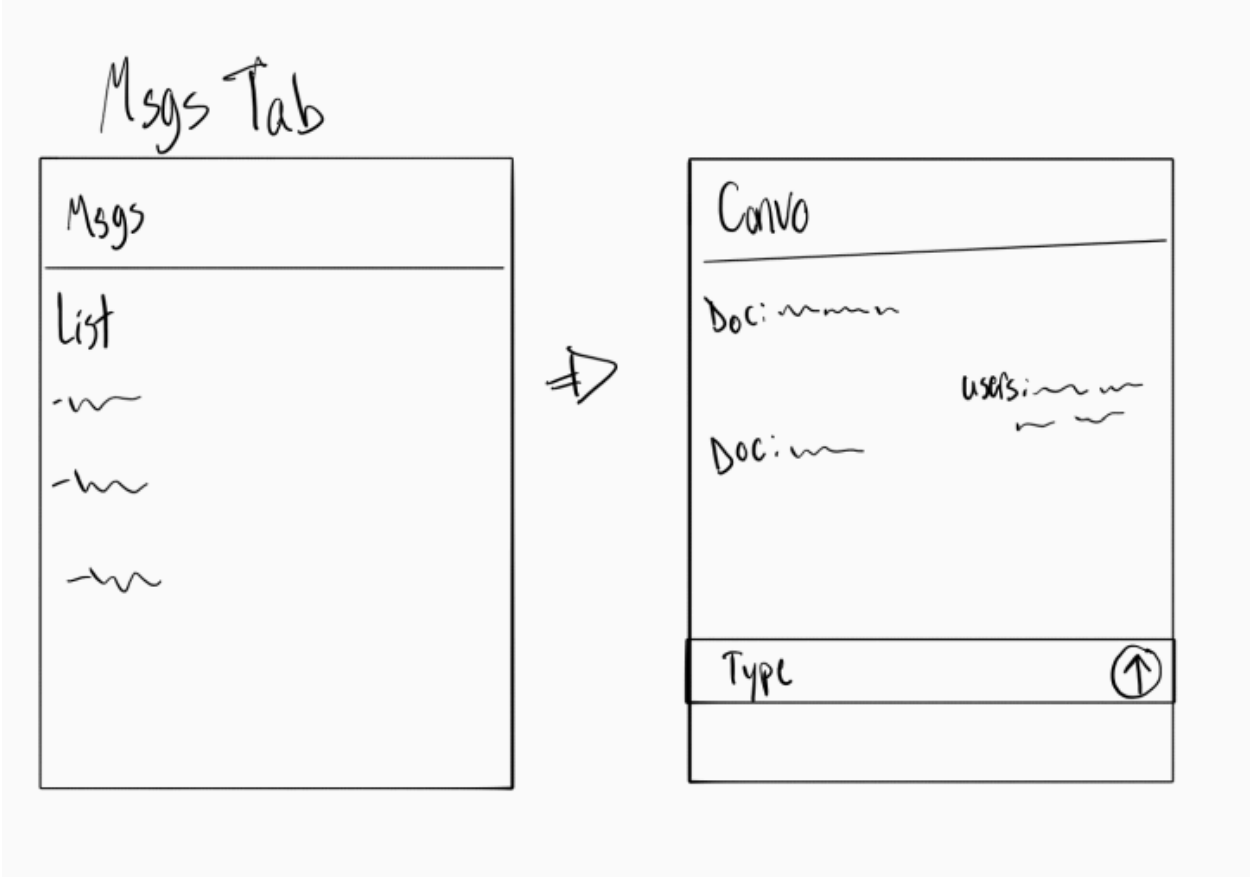
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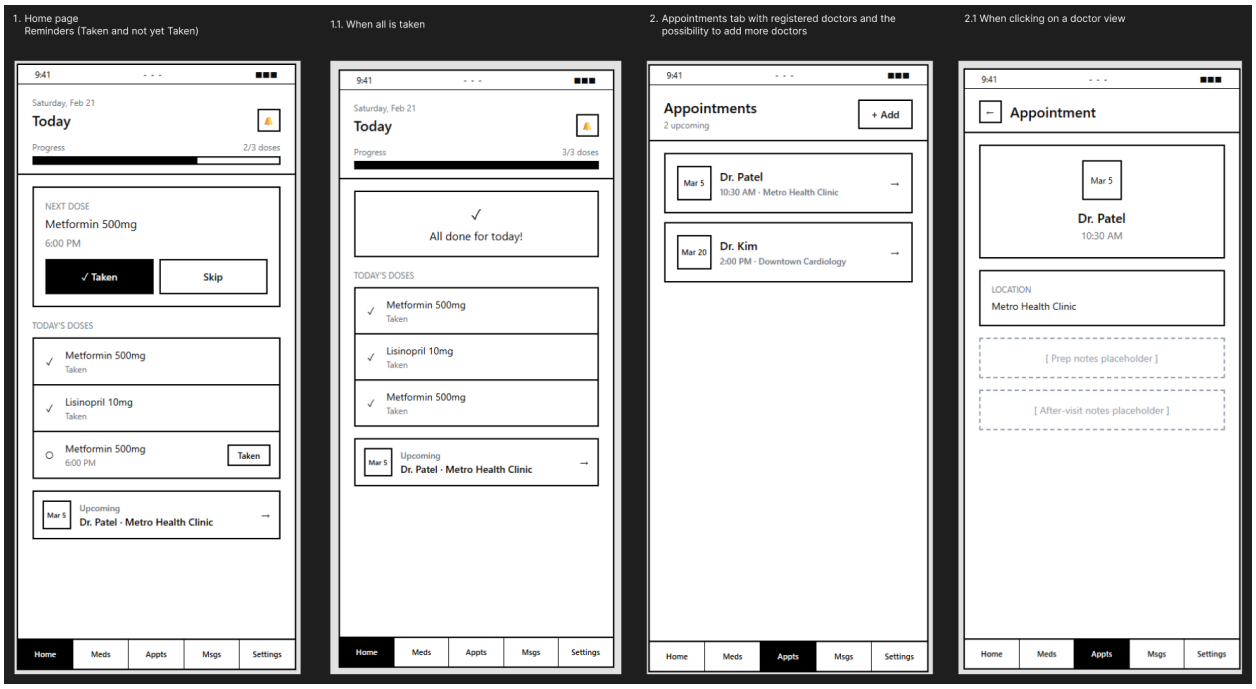
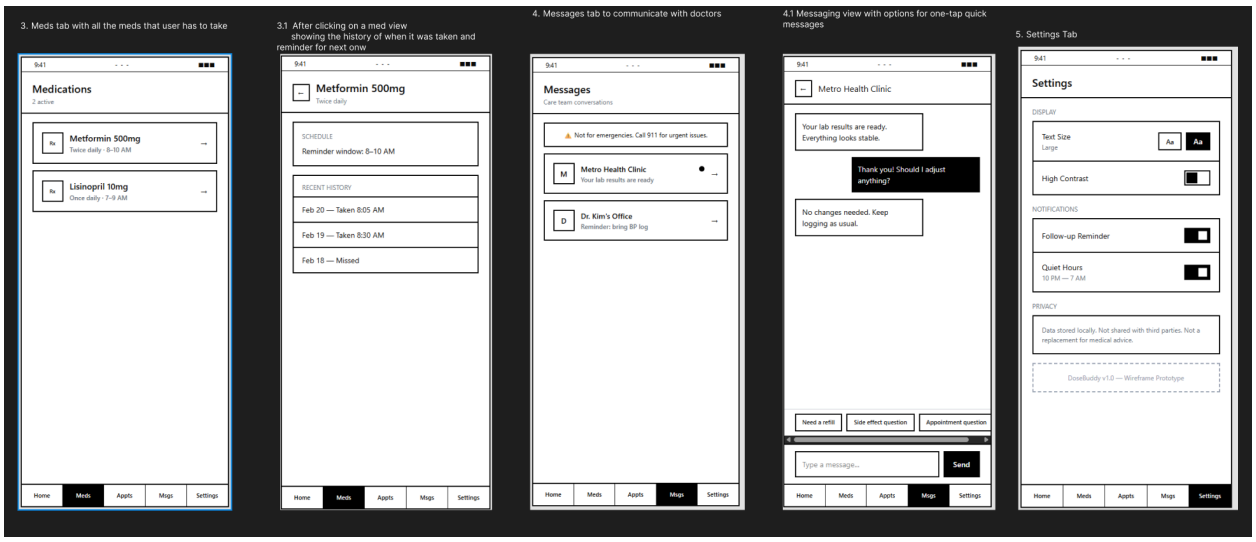
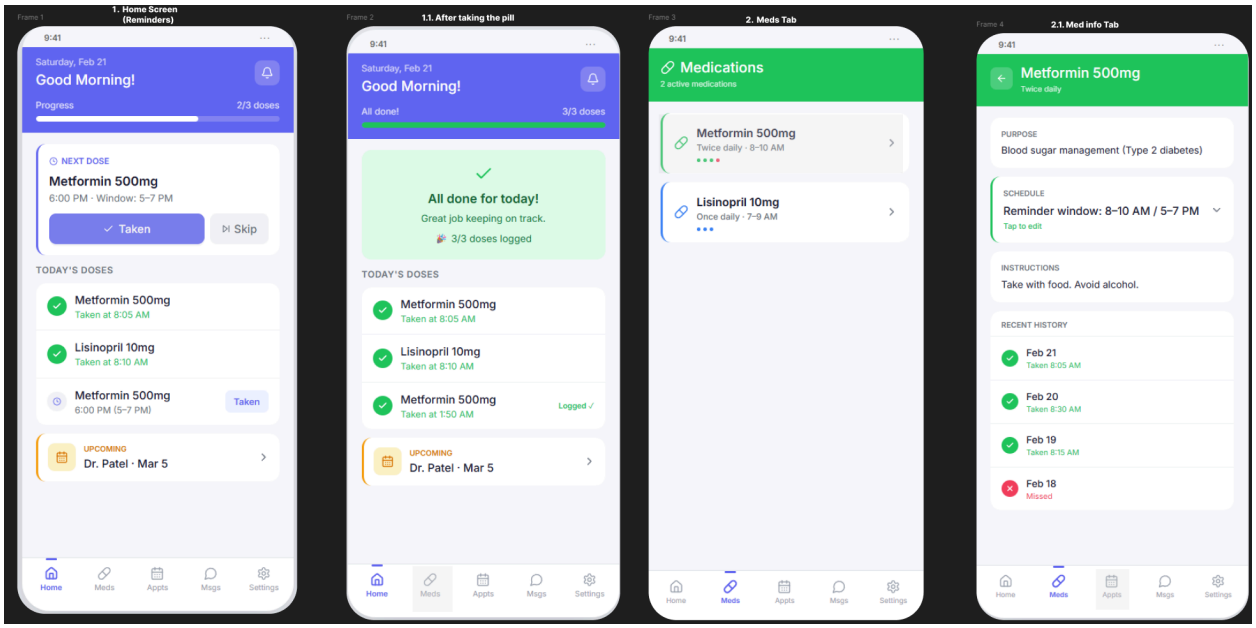
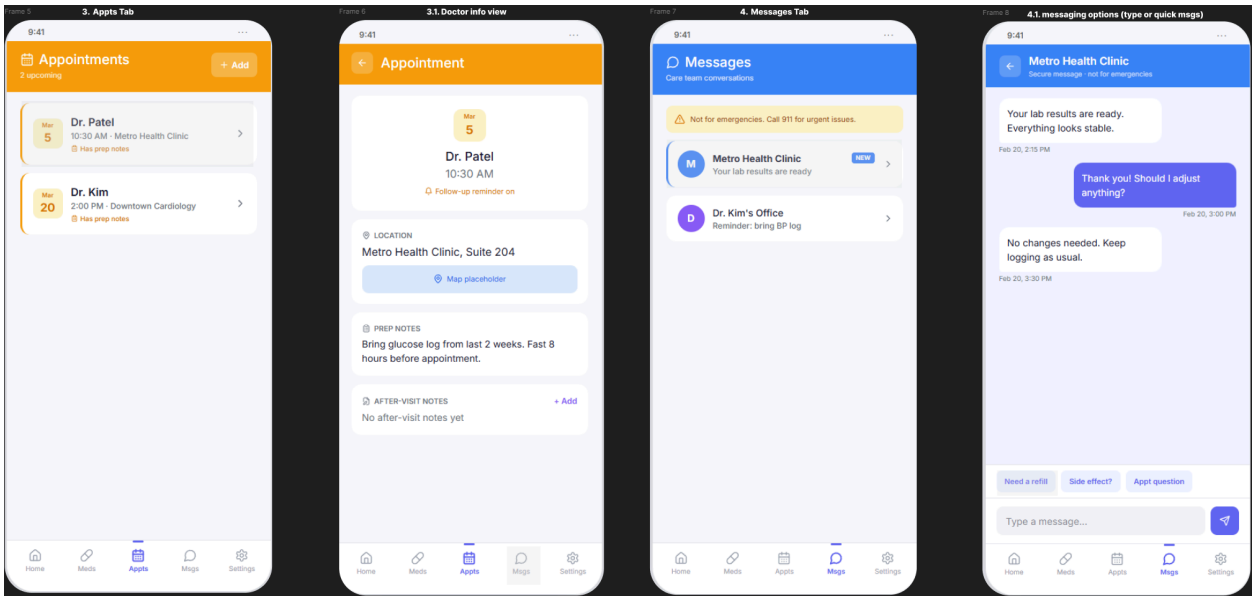
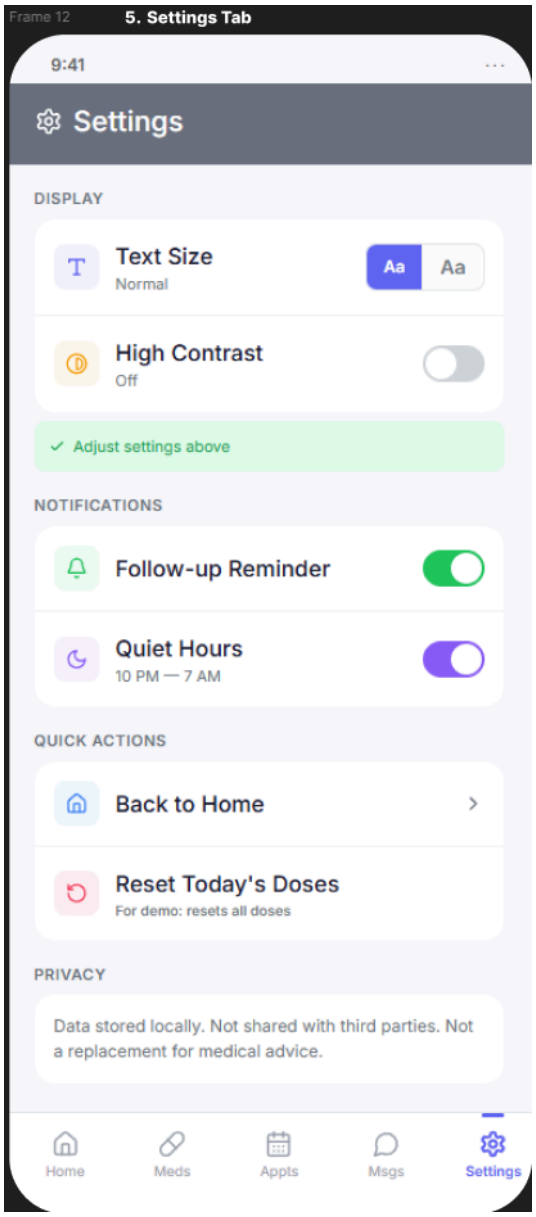




7.3. Prototypes







Visual Style

DoseBuddy follows a calm, card-based design language. Every screen is built around one primary action and one clear piece of information, nothing competes for attention. Cards are well-spaced with generous padding, and each section is clearly separated so the user always knows where they are and what they're looking at. This was a deliberate response to the research finding that cluttered layouts cause users like Noha to lose trust in an app before they've even used it properly.

Colour

One of the more distinctive choices in DoseBuddy is that each tab has its own colour identity, all sharing the same soft, muted tone:

- **Purple**: Home tab, primary actions (Taken button, progress bar)
- **Green**: Medications tab, confirmed/taken states
- **Orange**: Appointments tab, upcoming visit cards
- **Blue**: Messages tab, conversation threads
- **Grey**: Settings Tab

This system does two things at once: it gives users an immediate visual cue about which section they're in, and it creates clear emotional associations, green means done, orange means upcoming, blue means communication. The "All done for today!" The success state leans fully into green, which feels rewarding without being excessive. The background stays neutral light grey throughout with white cards, keeping the hierarchy readable at a glance.

Typography

Headings are large, bold, and clearly weighted above supporting information. Timestamps, secondary labels, and helper text are visually de-emphasised through lighter weight and smaller size, but remain readable. Every navigation icon has a text label underneath, which was a direct design decision coming out of the research: icon-only navigation was one of the most common frustrations among lower-tech users.

Animations & Interactions

Transitions are kept subtle and quick, the priority is feeling responsive, not decorative. The most important interaction, logging a dose, gives instant visual feedback: the timestamp appears immediately in the Today's Doses list and the progress bar updates, so the user knows their action registered without needing an extra confirmation screen. The "All done for today!" state is the one moment where the design allows itself a bit more expressiveness, a full-screen green confirmation that gives the user a small but satisfying sense of completion.

In addition, an interactive prototype has been developed to simulate how the app would behave in different scenarios:
https://www.figma.com/proto/ITUWwoLbCP093SMIQShCOV/357-mini-project?node-id=2-95&p=f&t=VW3IY2bRsn4EVEZz-1&scaling=min-zoom&content-scaling=fixed&page-id=2%3A2&starting-point-node-id=2%3A6

8. Usability Testing

8.1. Testing Plan

Purpose

The goal of usability testing would be to verify whether DoseBuddy actually feels fast and clear in practice, particularly for the two main persona needs: Marcus, who needs speed and one-tap actions to log and confirm quickly, and Noha, who needs clarity and reassurance to understand what's happening and avoid mistakes.

Testing Goals

- Verify that users can log a dose quickly without confusion
- Confirm that users can answer "Did I take it?" using the home timeline
- Check whether users can find and understand appointment details such as location and prep notes
- Assess whether messaging feels low-pressure and intuitive rather than like an emergency channel
- Evaluate basic accessibility clarity across labels, readability, and navigation

Method

The test would be conducted as a moderated usability session using the think-aloud protocol, where participants narrate their thoughts as they interact with the prototype. Five participants would be recruited, representing a mix of medium and high tech comfort levels, with at least one participant closer to Noha's profile in terms of lower tech comfort. Each session would run between 10 and 15 minutes, either in person or over a video call with screen sharing, using the clickable Figma prototype.

How Feedback Would Be Collected

- Observations and notes taken during each task, tracking where participants hesitate, misclick, or ask questions
- A single post-task confidence question: "How confident are you that you did it correctly?" rated on a scale of 1 to 5
- A short debrief at the end of each session covering what felt confusing, what felt helpful, and what participants would change

8.2. User Tasks

Participants would be given a short scenario and asked to complete each task without coaching. They would be encouraged to speak out loud throughout.

Scenario: "You're using DoseBuddy to stay on track with your medications and appointments."

Task 1: Log a dose from the Home screen "Mark the next dose as taken." Success = the participant logs it in one tap and notices the timestamp update.

Task 2: Confirm whether a dose was taken "You're unsure if you took your evening dose. Check and tell me what the app says." Success = the participant uses the Today timeline or history log to answer confidently.

Task 3: Find the medication reminder window "Go to your medication details and tell me what time window the reminder is set to." Success = the participant locates the schedule and reminder window quickly.

Task 4: Check appointment details "Find your next appointment and tell me the location and any prep note." Success = the participant opens the appointment card and extracts the relevant details.

Task 5: Send a non-urgent message using a quick option "Message the clinic about a side effect using the quick message buttons." Success = the participant finds Messages, opens a thread, selects a template, and sends.

Task 6: Change text size in Settings (accessibility check) "Increase the text size in Settings." Success = the participant finds the option and understands how the toggle works.

For each task, the following would be recorded: time to complete, number of errors or misclicks, and confidence rating on a scale of 1 to 5.

8.3. Feedback Summary

Feedback would be analyzed by grouping participant comments and observations into recurring themes, then ranking each issue by severity.

How Feedback Would Be Analyzed

Repeated comments would be clustered by theme, for example, "button label was unclear" or "couldn't find the history." Each issue would then be assigned a severity level: High if it blocks task completion or risks a wrong action, Medium if it slows the user down or creates uncertainty, and Low if it is cosmetic or a matter of personal preference.

Expected Summary Structure

- What worked well across participants (2–4 observations)
- Pain points identified, ranked by severity (High / Medium / Low)
- Notable differences between faster, action-oriented users (Marcus-type) and more cautious, read-first users (Noha-type)

8.4. Design Iterations

Changes would be prioritized based on how frequently an issue came up and how severely it affected the experience.

Iteration Approach

High severity issues would be addressed first, particularly anything that could lead to incorrect dose logging or cause a user to lose confidence in the app. Medium severity issues would be tackled next, focusing on reducing unnecessary friction such as extra taps or unclear navigation paths. Low severity issues would be noted and flagged for later polish if time allowed.

Examples of Design Iterations That Could Result

- If users missed the timeline: make "Today's Doses" more visually prominent and add a clearer "History" label
- If users hesitated on "Skip": rename it to "Not now" or add short helper text clarifying that it does not log as taken
- If appointment prep notes were overlooked: replace placeholder text with a clearly labeled card titled "Prep Notes"
- If messaging felt too much like a live chat: add visible expectation text such as "Replies may take 1–2 business days. Not for emergencies."
- If lower-tech users struggled: increase label size, reduce icon-only navigation, and add short helper text beneath primary action buttons

9. Conclusion & Reflection

DoseBuddy started with a straightforward question: why do people with chronic conditions still struggle to stay on top of something as routine as taking a pill? Going through the full UX process, research, personas, journey maps, wireframes, and a working prototype, made it clear that the problem was never really about memory or motivation. It was about friction. Too many steps, too much noise, and tools that weren't built for real life.

The UX process was what made the difference between designing based on assumptions and designing based on actual needs. Without the interviews, it would have been easy to build something that looked reasonable on paper but missed the point entirely. Talking to real people surfaced the specific pain points that shaped the whole design: the "did I already take it?" anxiety led directly to the timestamped home screen log, the notification fatigue led to lock-screen logging, and the frustration with scattered appointment details led to the structured appointment cards. None of those decisions came from guessing. They came from listening.

The persona work kept the design focused. Marcus and Noha represent genuinely different needs, and using them as anchors throughout prevented the app from becoming a feature dump. Every screen had to justify itself against at least one of them.

The biggest challenge during the process was balancing the needs of two very different users without building two different apps. What works for Marcus, fast and minimal, risks feeling too sparse for Noha. What reassures Noha, clear labels, more guidance, slows Marcus down. The solution was to make the core interactions identical (one tap, one glance) but layer in accessibility options through Settings, so the same app can serve both without compromise.

Another challenge was scope. It was tempting to keep adding features, a refill tracker, a symptom log, a caregiver mode. Keeping the design focused on the core loop (log, confirm, prepare, communicate) required actively cutting ideas that were interesting but not essential. The personas helped with that too.

If anything were to be revisited with more time, onboarding would be the priority. The current prototype jumps straight into the home screen, which works for demo purposes but doesn't reflect what a real first-time user would experience, especially someone like Noha. A proper guided setup would meaningfully improve the experience for that persona.

Overall, the process confirmed that good UX isn't about adding more. It's about removing every obstacle between the user and the thing they actually need to do. For DoseBuddy, that means logging a dose in one tap, confirming it in one glance, and never making a user feel like they're doing something wrong.